APSS321 Abnormal Psychology 变态心理学

Wk 1

Abnormal Behavior

Definition 4Ds: Dysfunctional, Distress, Deviance, Dangerous (additional criteria)

Dysfunctional: make well-being difficult to obtain, disturbs & deteriorates in cognition emotion, behavior, functions and job performance

Distress: Experiences disabling condition in social, occupational, other, may psychological or physical pain

Deviance: move away from norm / mean / infrequent behavior (change across culture and time, e.g., homosexuality, shamanism), Atypical (such as naked run) != Abnormal

Dangerous: Threat to the safety of this person (or others), e.g., suicidal ideation/attempt, violent attempt (anti-social disorder), mental disorder != dangerousness

Controversial people may show some deviant behaviors but need comprehensive assessment for other Ds (e.g., Trump)

DSM-x = Diagnostic and Statistical Manual of Mental Disorder

Abnormal psychology vs psychopathology: abnormal behavior can lead to mental illness

Mental health issues: 973M people worldwide, one in seven in HK common mental disorder, 1 in 10 young people major depression

50% of mental health disorders show first signs before a person's 14 years old, 75% begin before 24. (Adolescent interference is important!)

People with mental health problems: Over 10x more likely to be victims of violent crimes than general population

Mental health != being lazy or weak, can get better and many recover completely

Understanding of mental illness changes with time

Mental Disorder Overview

Prehistorical views: Supernatural theories, trephination (环锯术 surgical drilling of holes in skulls), exorcism 驱魔

Greco-Roman thought

Hippocrates 460-377BC: physical ailments, imbalance of humor (blood, black/yellow bile, phlygm)

Plato 429-346BC: have the responsibility to care for mental disordered people

Asclepiades & Cicero (~100BC): melancholy arises from grief fear and rage, consider balance in the physical and mental domain

Middle Age (500~1500 AD)

Rise of supernatural theories, Sci and med discarded due to power of the church

Mental illness: possession by the Devil (burn people)

The Renaissance (14-16c): Rise of Humanism, human wellness (by locking them in hospital / Asylums, dungeons)

Reform Movement

Moral Treatment Movement: humane treatment, respectful and moral guidance, outside exercise, better room, removal of chains and cuffs

Mental Hygiene Movement 18-19c: prevention by social measures (education, decent workplace & living, psychiatric services)

Actual situation: YT-EtgzpAbxcRI Behind Closed Doors: A Look Inside Insane Asylums of the 19th Century

20-21c Biological somatogenic perspective

Emil Modern scientific psychiatry, textbook became the basis DSM, syndrome not symptom

Cooccurrence between physical illness and mental illness

Benjamin Franklin: accidentally found that severe electric shock changes one's memory (still works nowadays)

Psychological/Psychogenic perspective: emotional or psychological factors are the cause of mental disorders, hypnosis as a treatment by Jean-Martin Charcot and Pierre Janet

Current - Biopsychosocial model

Various causes: biological psychological and social-cultural perspectives

Develop and use of psychiatric or phycho drugs

Multicultural psychology: take into account the trates and backgrounds of patients

APA 2002: Six guidelines

Prevalence

19.9% US adult – experienced mental disorders in 2019

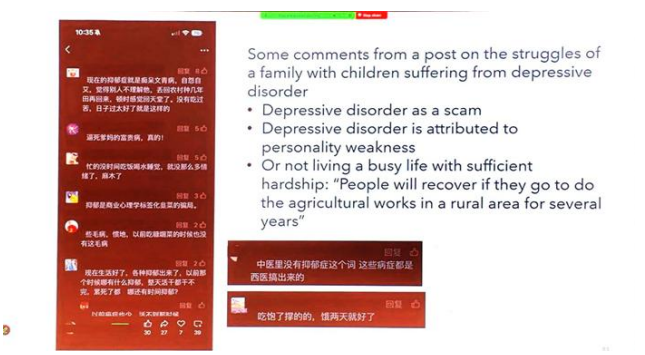
57.2% of adults in the US did not receive treatment

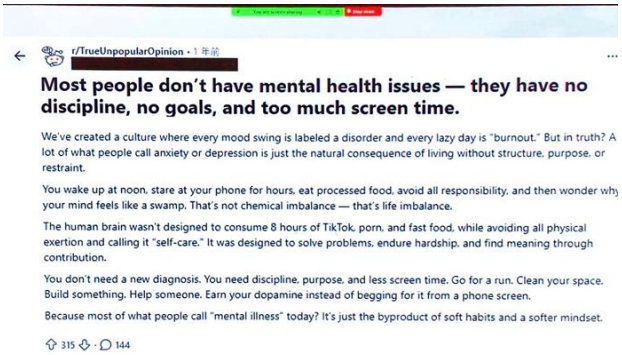
US: Major depression 17% alcohol abuse 13% Social anxiety disorder 12% Conduct disorder 9%

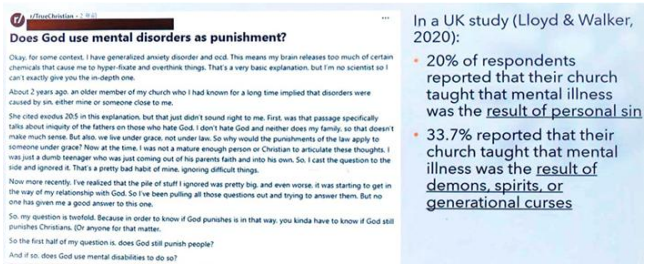
HK: anxiety disorder 8%, mood disorder 4.3% OC-and-related 3.7% PTSD 3.4% Major depression 2.8%

Deinstitutionalization: closing large asylums and integrating community, treated locally

Cost of Mental Illness: US 193M USD earning lost, 2.5T$ globally in 2010, 6$T by 2030, 37% students drop out of school







mental health stigma (YouTube: VQoiz4wfV_c / 9cqNe97I2Bs social experiment on mental health stigma)

you can be born perfectly rich have a very good lifestyle have parents who care very well for you

i don't think mental health issues discriminate against anyone

……

if possible i would just stay away as far as possible, let's go sell it to house

uh i didn't introduce myself fully

i suffer from clinical depression. i was clinically diagnosed with depression

i'm living in recovery, and i overcame it i wrote a letter that

i would like you to read out to me a lot

[Music]

12 years is a long time to live with a mental health condition
in the 12 years that have been diagnosed with depression
this is what i've learned
i've learned that the greatest battles we fight are the ones with ourselves
i've learned that most conversation about employment and the moment i review my diagnosis
i've learned that shame is something society teaches you
i've learned how to say sorry for showing symptoms of my condition
i've learned that friends can disappear and i will never hear from them again
[Music]
i've learned that family doesn't always know how to support you
no matter how much you need them
i've learned that hope is a little voice a whispers maybe
i've learned that recovery is a process
i've learned that one small crack doesn't mean i'm broken
i've learned that i have an illness but this illness is not my identity
so sorry that you have to go through this
[Music]
i'm so sorry
[Music]
do you have any questions you want to ask me

Public-Stigma (污名), Self-stigma, Stigma Expectations => label avoidance, courtesy stigma

Suggestions from NAMI: Talk openly about mental health, educate yourself and others with facts, reminder to be conscious of language, encourage equality (draw comparisons with those with cancer), show compassion, be honest about treatment, let the media know, choose empowerment over shame

Change the social norms of pluralistic ignorance against mental health

Gaming and immersive experience-based interventions

Session 2

***Peter's Case**: **Peter Red** is a 17-year-old boy in Grade 12. He was the captain of his football team and has always been considered a good student in the eyes of his teacher. His peers describe him as an agreeable and friendly person. However, Peter was involved in a fight with his teammates lately. He also had sleep problems and lost 15 pounds over the last two months. Peter was diagnosed with a major depression order. In the clinical assessment it is known that people's father is also suffering from chronic depression and receiving treatment. The clinician further found that Peter had a bad relationship with his parents lately.*

Theoretical Models of Abnormality

Multidimensional model

Biological Approach:

Genetic Contribution (e.g., CACNA1C, especially ADHD Autism but not for other disorders <50%),

Nature vs Nurture: gene-environment interaction (e.g., diathesis-stress model  ; Epigenetics: may activate/turned off by env beh influence, YT: _aAhcNjmvhc; nongenomic inheritance of behavior; cross-fostering study: rats born to easily stressed mothers but reared by calm mothers grew up more calm and supportive)

Neuroscience Contribution

Neurotransmitters: Dopamine (voluntary movement and reward mech => motivation), Serotonin (pain, sleep cycle, digestion, => stable mood), Endorphins (reduce pain => calm and happy), norepinephrine (heart rate and blood pressure => mood and emotional responses to stress and fear), GABA (block excitatory transmitters => reducing the anxiety and panic), glutamate (learning & memory)

Imbalance of chemicals in the brain, e.g., Too much / incorrect dopamine => hallucination; wintertime inadequate Serotonin => Seasonal Affective Disorder (SAD)

Brain Function: Thalamus (sense), Hypothalamus (autonomic nervous system ANS), Amygdala (sensory info & determine emotional importance), Hippocampus (gateway to memory)

Brain plasticity: "I only have half a brain" YT: f2fCY_M7Vms

Psychological Approaches: Psychodynamic, Behavioral, cognitive, Humanistic and Existential Approach

Psychodynamic: Unconscious desires and childhood experience influence our behavior

e.g., 5 psychosexual stages: oral (0~1 mouth, weaning of breast or bottle => smoking, overeating) -> Anal (1-3, Anus, Toilet training => Neatness/Messiness) -> Phallic (3-6, Genitals, Oedipus / Electra complex => Vanity,

exhibitionism) -> Latency (6-12) -> Genital (12+)

Id (unconscious, operates on pleasure principle) vs Superego (partly conscious, operates on internalized moral principles and societal rules) vs ego (partly conscious, rationalize a balance between id and superego)

Defense mechanism: echo seeks to restore the feeling of anxiety resulted from the conflict between id and superego (e.g., stress/dissatisfaction to boss: displace to family members, sublimation by Boxing, repression & regression), associated with mental disorder when used excessively or problematically

Behavioral Approach: learned, and mental disorder is also a consequence of learning

2 types: Conditioning (classical/operant, from personal experience) and Observational/social learning

PTSD (2001-9-11 -> plane)

Cognitive Approach:

Information-processing model: input (attention, by sensory & memory into short term working memory) -> processing (encoding) -> storage (long-term memory) -> retrieval -> output (emotional, physiological & behavior)

Cognitive distortion by Aron Beck (1921-2021): irrational and dysfunctional thought patterns -> psychopathology

Distorted cognitive processes: selective abstraction, minimization, personalization, arbitrary inference, magnification, overgeneralization, dichotomous thinking (二分法思维), inability to disconfirm

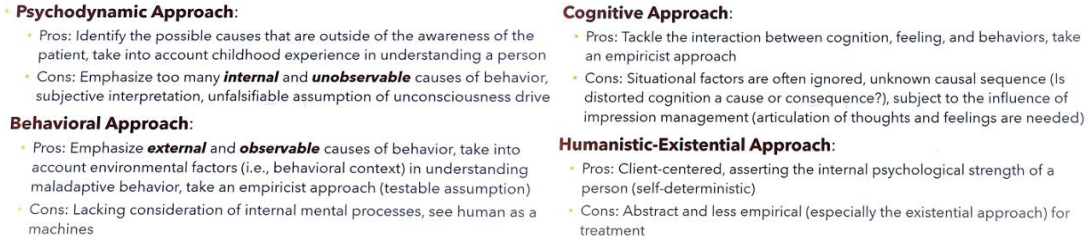
Humanistic Approach: unconditional positive regard from significant others in their life

Conditional positive regard -> distort in self-concept

Existential Approach: fundamental human needs

Existential anxieties (everyone has!): isolation, loss of freedom, meaninglessness

Emphasize the importance of: Interpersonal relationship, free will (personal choice), meaning of life



Sociocultural Model: sociocultural factors such as gender, social class, ethnicity, culture, quality of social institutions

Environmental stressors: stressful environment for indi, unemployment, poverty, family breakdown, casualties

Socialization experience: socialization = the process an individual becomes an adaptive member of society,

partially shapes people's values/beliefs/attitudes/behaviors, creates systematic differences between people

Behavior can be adaptive in one culture but not in another, Culture alters expression of preference/emotion

e.g., an introvert person living in a community with a strong party culture => require person-culture fit

Stigmatization 污名化: negative social norm against mental illness → makes mental health a taboo, people can't talk about it

cultural bound syndrome: psychopathology that appears in one culture but not in the other, e.g., Susto, Taijin kyofusho

Dr. Oz On Why Men Are Embarrassed To Talk About Mental Health Issues YT: LvWpxS_hgSA

Biopsychosocial model: Biological, psychological, sociocultural

Equifinality 等效性: different pathways can cause the same disorder

Research methods

Scientific Field: A systematic method for gathering knowledge about the world around us

Research literature <-> RQ -> empirical study -> data analysis -> conclusions

Observational Methods

Naturalistic observation (no potential influence of experimenters / lab settings, but takes too long time)

Lab observations (can use more sophisticated equipment to record the session and have more flexibility for manipulation, but may change people's behavior that deteriate generalization)

Case studies (careful observation, in-depth interviews, personal docs such as diaries, personal notes, official docs, can provide detailed info and in-depth understanding a case/beh especially those rare, but may not be generalizable and subject to researcher's bias, and time consuming and difficult to replicate)

Survey Method: Usually questionnaire, describe or correlational research

ABAB design: A1 no treatment -> B1 receive treatment -> A2 remove treatment -> provide treatment again

Activity: How to get the factors of depression development in the population

https://docs.google.com/document/d/1MeN8nTRjgvPCllUW3_DylTmzU3aREQm-BteJ1eCVKWg/edit?

Survey Method

- **Pros**: Anonymity, larger scale, can find some common trends in upbringing/ life experiences and family background/ other factors hypothesised to affect the likelihood of depression development
- **Cons**: Social desirability bias, subjective rating, difficult to get detailed answers, still not conclusively causational

+Interview/ Case study – to get further elaboration/ clarification (qualitative data)

Survey method: distribute questionnaires to people with no depression and depression. Compare the differences between different factors (eg, self- concept, family, environmental factors) and establish correlation between how they rate their current situation with the depressive symptoms that they have.

Explore and generate hypotheses with CASE STUDY, then test factors across a population with SURVEY METHOD, further test with EXPERIMENTAL RESEARCH for the treatment causality.

Longitudinal survey research

1. Identify potential independent variables and formulate hypotheses, such as "Low self-esteem and low social support lead to increased future levels of depression."
2. Sample and track participants, using long-term, multiple trails to detect scores on independent variables and depression scores.
3. Data analysis is conducted to verify whether the identified variables actually lead to increased depression scores over time.

Correlational research methods such as case studies to study particular factors in depth, also uses survey to collect large amount of data to analyse,

Session 3 Clinical assessment

Purpose: diagnosis (criteria, DSM-5 ICD-11), etiology (search for the cause), treatment, progress (recovery), communication

Process: like a funnel — collect a wide range of info (environment, response, functioning) → rule out irrelevant areas → concentrate on the most relevant; start with general social & psychological functioning

Assessment vs testing: assessment is problem-oriented (decision making), psychological testing is measures psychology-related variables (a tool to make clinical assessment)

Key criteria for evaluating assessment: Reliability, standardization, validity (concurrent/descriptive. Content/face validity, predictive validity)

Methods

Clinical interview: normally face-to-face (verbal and non-verbal), semi-/un-/structured format, with the purpose of in-take -> diagnostic -> for etiology 病因 -> follow-up -> end-of-therapy

Mental status examination: appearance and behavior, sensorium (dis/orientation) and cognition (consciousness, attention, memory, language), mood and affect (subjectively reported + objectively observed), speech (rate, volume, fluency), thought process and contents, perceptual disturbances (sense something that others do not sense), insights and judgements

Structured / semi-structured interview

Based on standardized protocols (how rigidly the interviewer must follow is different). Usable by non-professionals only with sufficient training. Developed from prior experience/pilot trials.

Strengths: higher diagnostic accuracy & reliability with consistent results; good for training new clinicians; focused on specific disorders → helps differential diagnosis (get the DSM-5 criteria info to build a case).

Limitations: focuses only on the problem; mechanistic (weakens rapport); only covers what the protocol/diagnostic criteria include; long administration time

Unstructured interview: YT: G1CqpUI6ntQ

Strength: More friendly and empathetic to build up rapport, flexible, more in-depth info, care for not only symptoms or factual info but also the presentation of respondents

weaknesses: may miss some essential elements, inconsistent quality

Computer Assisted Interviews

Strength: Avoid embarrassment or unwillingness to share, less conscious, more patient to complaints, less costly

Limitation: lack human touch, difficult to build bond, cannot detect non-verbal

Behavioral assessment: behavior is the central target of evaluation

Target behavior = behavior to be changed, can be in excess or deficit state

ABC model: antecedent of behavior -> behavior -> consequence of behavior

Behavioral interview: unstructured (e.g., done in class activity) / structured

Observation, in naturalistic (high ecological validity in real world but hard for infrequent/dangerous behaviors and can't manipulate environment) vs analog settings (lab, test causal hypotheses, role-play, exposure; observe infrequent/dangerous behaviors with consent + safety)

Participant observers (family, coworkers, friends — already in client's daily life): most unobtrusive (behavior less likely to change); observe across multiple settings & time points. But with Limitations that limited focus (other duties), subjective bias, may lack expertise to interpret

Non-participant observers: such as experts/clinicians/researchers

Behavioral rating scales (e.g., Questions about behavioral function QABF)

Self-monitoring: assess naturalistically, infrequent/hidden behaviors, but reactivity (behavior change when being assessed), potential consequences, unaware of behavioral occurrences

Psychological Tests and Inventories

Two paradigms: G-factor (general intelligence) vs multiple intelligences

Intelligence test (->IQ): Stanford-Binet intelligence test (g-factor), Wechsler Adult intelligence scale (multiple intelligence), relative score, originally for screening students needing special assistance, not to judge "how smart"

Personality test (projective/objective personality test):

Projective 投射性 (psychoanalytic/psychodynamic tradition): personality largely unconscious → project onto ambiguous stimuli via free association, but have the risk of cultural bias. Tests: Thematic Apperception Test TAT, Rorschach inkblot, sentence completion

Objective (empirically/psychometrically based): scored from responses to statements (feel/believe/behave)

Projective vs objective: projective = freer expression, taps subconscious/unconscious but low reliability/interpretation bias; objective = standardized scoring, reliable/valid but limited to preset items, social desirability/impression management, response set bias, memory bias

MMPI-2 clinical scales:

Hypochondriasis — many physical complaints, cynical defeatist attitudes, perceived as whiny/demanding.

Depression — depressed mood, pessimistic, worrisome, despondent, lethargic (reduced interest, sleep disturbance).

Hysteria — naïve, egocentric, little insight, immature; stress-related physical complaints.

Psychopathic deviate — difficulty incorporating society's values, rebellious, impulsive, antisocial tendencies (rule/social-norm violating).

Masculinity-Femininity — identifying with opposite traditional gender role (dated, but reflects preferences/stress context).

Paranoia — suspicious, guarded, blames others, resentful, aloof, distrust.

Psychasthenia — anxious, fearful, tense, worried, insecure, difficulty concentrating, obsessional, self-doubting.

Schizophrenia — confused/illogical thinking, delusions, hallucinations, feels alienated/misunderstood.

Hypomania — unusually energetic/active, inflated self-image, unrealistic plans, pressured/rapid speech.

Social Introversion — shy, dislikes parties. - MMPI validity scales: L (Lying / fake-good), F (Infrequency / fake-bad), K (Defensiveness) — detect faking.

Response inventories (self-report on one specific function, e.g., social anxiety, emotional skills)

Strengths: high (face) validity; comprehensive picture of one area.

Limitations: guessable → social desirability; many lack standardized norms (no reference to interpret absolute scores). e.g., Beck Depression Inventory (based on diagnostic criteria; provides score-interpretation guidelines

Neurological/neuropsychological:

Neurological test = brain scan, examines structure (abnormalities) and activity/function.

CT/CAT scan: structural abnormalities via tissue-density differences.

MRI: similar, higher image quality.

fMRI: function/activation via blood flow; baseline vs stimulus-triggered activation.

PET scan (positron emission tomography): chemical activity, uses injected tracer.

(YT: ng3qXfck6M8) Brain scans can assist diagnosis (e.g., depression reduced activity) but not sufficient alone — need multiple evidence.

Neuropsychological test: behavioral task assessing brain function indirectly (integrated cognitive-perceptual-motor function). Used to detect/recover cognitive impairment (e.g., delirium — temporary cognitive deterioration from intoxication/illness). Example: Bender-Gestalt test (copy figures; occipital-lobe damage → difficulty).

Strengths: high validity & reliability. Weaknesses: costly; interpretation needs trained professionals.

Mental Disorder Description

Terms & glossaries

Prevalence: Point (active cases at a point in time)/Period (during a period, usually last 12 months)/Lifetime Prevalence (any time in life)

Incidence: number of **new** cases over a period

Comorbidity: Co-occurring of 2+ mental disorders

Etiology 病因: cause of disorder

Onset 发病: Acute (sudden, intense), insidious (gradual, e.g., schizophrenia)

Course: particular pattern of disorder: episodic (bipolar, MDD), chronic (persistent), time-limited (brief psychotic disorder).

Prognosis: Anticipated course the mental disorder will take, e.g., changes with age — hoarding worsens

2 systems: Diagnostic and Statical Manual of Mental Disorders (DSM), International Classification of Diseases (ICD)

DSM change: moved away from the multi-axial system (DSM-III → III-R → IV → IV-TR) to an empirical (observation-based) system in DSM-5:

Removed multiaxial → aligned with ICD (psychiatric/medical dx + psychosocial/contextual factors + disability, replacing GAF).

Categorical → dimensional (levels of severity). e.g., alcohol abuse/dependence combined into alcohol use disorder with mild/moderate/severe by symptom count.

Elements of diagnosis: Diagnostic criteria and descriptors (criteria), subtypes and specifier (non-/mutually exclusive), Principal Diagnosis, Provisional Diagnosis

Diagnostic criteria & descriptors (description, course).

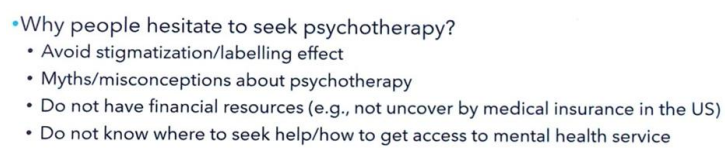
Subtypes — different representations/causes, mutually exclusive (e.g., non-REM sleep arousal: sleepwalking or sleep-terror type).

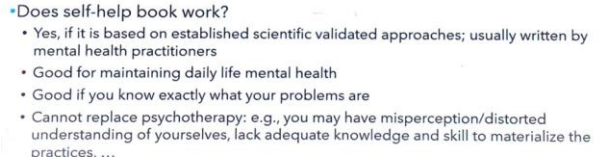
Specifiers — non-mutually exclusive (e.g., bipolar with anxious-distressed + melancholic features, any combination).

Principal diagnosis — the reason for admission/focus of treatment.

Provisional diagnosis — insufficient info yet, but strong assumption the full criteria will be met (e.g., neurocognitive disorders).

Psychotherapy: scientifically validated procedures, emphasis on client-therapist relationship, incl. Psychoanalysis/psychodynamic therapy, BT/CT/CBT, person-centered therapy





The remaining: Different Mental Disorders: {features of mental disorder, common disorders, common tratement}

Anxiety Disorders

Excessive/irrational future events / fears (immediate alarms to danger, may lead to tendency to escape), panic attacks

Anxiety/fear become a problem when: levels too high (past the optimal arousal level — Yerkes-Dodson inverted-U: too low = unmotivated, too high = task-disrupting physiological discomfort), or false alarm — response disproportionate/excessive relative to actual danger = clinical anxiety

Can be cued (triggered by specific objects/types) and uncued, Come on suddenly, usually peak at 10 mins,

Most prevalent: **specific phobia**, then social anxiety. All more prevalent in **females**. Onset varies: childhood, adolescence, young adults, older adults

China prevalence: 5% 1-year, 7.6% lifetime, Female>Male

Symptoms of Anxiety and related disorders: GAD OCD PTSD, panic disorder, …

Signs of a Panic Attack, YT: Ya1HfNY7898

Cause

Genes: heritability ~30–40%; serotonin 血清素 transporter gene → reduced serotonin activity → anxiety-related personality traits.

Neurotransmitters: poor serotonin functioning; high/abnormal norepinephrine (fight-or-flight → arousal).

Fear circuit (recap): sensory info → thalamus → amygdala (evaluates threat) → hypothalamus → endocrine + autonomic (fight-or-flight) + cortex.

Hypersensitive fear circuit: higher baseline arousal; arousal shoots up immediately; longer/higher plateau; harder to down-regulate.

Sensitive amygdala → HPA axis (adrenaline, cortisol).

Brain regions:

ACC (anterior cingulate cortex) — attention control; attention bias toward threat.

vmPFC (ventromedial prefrontal cortex) — filters threatening vs non-threatening; weak filtering → constant activation.

Hippocampus 海马体— registers experiences into long-term memory.

Basal ganglia 基底神经节— conditioned rumination/compulsions (OCD).

Hyperactive locus coeruleus — controls norepinephrine on/off; never turns off → chronically high arousal, panic-like symptoms

Common Mechanisms of anxiety disorder:

- behavioral perspective: Classical perspective, stimulus generalization, social learning
- sociocultural: environmental stressors that triggers chronically higher levels of anxiety and fear, statistically on gender F>M who tend to use emotion-focused coping vs addressing the problem itself

comorbidity: depression (~55% diagnosis correlation, ~76% if any time in life), physical disorder

Generalized Anxiety Disorder

Excessive and pervasive worry about everyday life events (>50% days in the past 6 months), Difficult to control the worry, Experience somatic symptoms 躯体症状 (3+ out of 6: Restlessness, tires easily, problem concentration, irritability, muscle tension, problem with sleep)

Possible causes: Biological Vulnerability (Hyperactivity anxiety circuits), Psychological Vulnerability, Sociocultural Vulnerability

Treatment:

- Medication such as benzodiazepines, buspirone

Behavioral and cognitive approaches: exposure to worry process, relaxation training (muscle groups, generating mental pictures of relaxation)

CBT: Intolerance of uncertainty therapy: tolerate uncertainty, identify & restructure cognition on potential threats or losses of control

Panic Disorder & Agoraphobia

Panic Disorder: Unexpected and **recurrent** panic attacks, worry about future attacks or losing one's mind (1 month or more), behavioral avoidance, may occur with or without Agoraphobia 广场恐惧症 (50% clinical Panic Disorder -> agoraphobia)

Prevalence: 2~3% within 12M across US and Euro, 4.7% lifetime, F>M

Agoraphobia: fear or anxiety about {2+ out of 5 of: using public transport, in open spaces, enclosed spaces, standing in line/being in a crowd, outside of the home alone}, where escape or getting help may be difficult, and where one almost always provoke fear or anxiety, out of proportion to the actual danger, typically lasting > 6M

Example: Woman's Struggle with Agoraphobia YT ihOyKq8ms58

Possible Psychological Cause: Catastrophizing misinterpretation of the physical sensation, frightening thought -> more symptoms -> more frightening thought -> ... -> full-grown panic attack

Treatment

Medication such as SSRIs SNRIs Benzodiazepines

CBT: psychoeducation, self-monitoring, relaxation training and progressive muscle relaxation, cognitive restructuring, exposure

Specific Phobia

Immediate Irrational fear of a specific object/situation that significantly interferes with one's ability of functioning, circumscribed to the presence of a particular situation/object, out of propotino to the actual danger, and tend to avoid

4 types: Natural environment / blood-injection-injury / Animal / situation phobia

Example: Can You Face Your WORST FEAR? Phobia Fighters Take the DEEP END! (BBC Explorations) YT BzNSYw7xwpU

Possible Cause: Psychological (Classical conditioning, observational learning), sociocultural (being told to be dangerous)

Treatment

Medication such as Benzodiazepines, Antidepressants (tricycles, SSRIs, SNRIs)

Systematic desensitization:

exposure-based exercises (e.g., Think about a spider -> look at the photo -> look at a real -> hold the box with it -> let it craw on the desk -> shoe -> pants leg -> sleeve -> arm -> OK). Example: Hypnosis Overcomes the Unthinkable - Phobias Cured Before Your Eyes (feather phobia) YT lMZ5o2uruXY

Flooding (intense exposure), Modeling (learn through observation)

Social Anxiety Disorder

(Almost always provoke) Extreme fear or anxiety and avoidance where one could potentially be evaluated negatively by others, which is out of proportion to the actual threat, persistent > 6M, and lead to one's avoidance or safety behaviors (less eye contact / interaction, asking questions off of oneself, corner position, bland neutral clothes)

Onset during adolescents

Classical conditioning (stressful or humiliating experience), maladaptive expectation and assumption, behavioral inhibition (presented with unfamiliar people or situations)

Treatment:

Medication such as SSRIs

Exposure (role-play -> real world setting), social skills training, cognitive restructuring

Obsessive-Compulsive Disorder OCD



Obsessions: thoughts and urges that are intrusive, nonsense, and unwanted, usually happen recurrently and persistently (Contamination, errors of uncertainty regarding daily behaviors, thoughts of physical harms), time-consuming or cause clinically significant distress or disfunction

Compulsions: repetitive behaviors or mental acts (handwashing, checking, counting, symmetry, repeating specific words)

OCD cycle: Obsessions -> anxiety -> compulsions -> belief -> obsessions -> …

Prevalence: 1~2%, F>M in adulthood and more cleaning related, M>F in childhood and more forbidden thoughts and symmetry related, mean=19.5-year-old

Possible causes: psychological perspective

psychodynamic: crash between id and superego (where compulsive behavior is a form of defense mechanism),

Behavioral: classical condition, operant conditioning,

Cognitive approach: disbelief of oneself, disconfirmation bias, over-interpret potential negative outcomes of events)

Behavioral-cognitive: thought-action-fusion (thinking about an action = carrying out the action -> pure obsession, leading to thought suppression)

Example: The Pandemic Through The Eyes Of Someone With OCD YT -Tk7cSyTmJk

Treatment of OCD

CBT: ERP (Relaxation training and hierarch of obsessions, exposure preventing to engage in compulsive behavior)

Related Disorder: Body Dysmorphic Disorder 躯体变形障碍, Hoarding Disorder 囤积症,

Posttraumatic Stress Disorder (PTSD)

Stressors

Chronic stressors 慢性压力: events that persist over an extended period of time

Acute stressors 急性压力: brief focal events sometimes continue to be experiences as overwhelming well after the event has ended

Daily Hassles: minor irritations and annoyances

Eustress 良性压力: positive life events that cause stress (e.g., marriage, getting promoted that requires adjust to new life/workstyle: social readjustment rating scale SRRS)

Trauma: actual or threated death or serious injury, e.g., Non-white, low SES

PTSD

First known as shell shock or combat neurosis back in WW1

Directly exposed to or witnessing or occur on a closed family/friend extremely stressful or traumatic events (e.g., 911 attack GRAPHIC WARNING: Photographer Shannon Stapleton on 9/11 anniversary and PTSD YT 9LNg8J-SE2E)

4 categories of symptoms (at least 1M):

Intrusion symptoms: recurrent involuntary and intrusive distressing memories of the traumatic events

Avoidance of stimuli liked to the event (locations, individuals, activities, or specific situations)

Negative alternations of cognition or mood: Impaired memory about the traumatic event 记忆受损/解离性遗忘, exaggerated negative beliefs of expectation towards oneself/others/world, distorted cognition about the cause of the event-> guilt (e.g., 幸存者内疚 survival guilt), persistent negative emotions, weakened interest / motivation, Distance oneself from others, unable to experience positive emotion

Alterations in arousal and reactivity

Prevalence: 3.5% in the past 12M, lifetime risk 8.7%; 0.2% among Chinese adults; F with a longer course (but no gender difference exists following the same traumatic event); onset typically occurs in the first few months after trauma, with delayed onset at 6 months

Comorbidity: ~80% present with depression, bipolar disorder, anxiety, or substance abuse

Possible Causes

Biological: HPA axis and fear circuit hypersensitivity (amygdala 杏仁核 activation → stress hormone secretion → heightened physiological arousal)

Psychodynamic: Symptoms serve as defense mechanisms (repression/dissociation to avoid unbearable reality)

Behavioral: Classical conditioning - traumatic events inherently elicit fear; surrounding neutral stimuli become CS that trigger distress (CR)

Cognitive: Rumination/over-analysis of negative events, perception of events as uncontrollable, and a tendency toward self-blame

Existential: Ultimate threat to survival combined with a sense of helplessness that persists after the event has ended

Sociocultural: Trauma exposure rates vary across cultural groups; individualistic cultures favor internal attributions (leading to self-blame and higher PTSD risk/prevalence, e.g., US > China), whereas collectivistic cultures favor external/situational attributions (though socially stigmatized events like sexual assault still heighten stress)

Treatment

Medication: Anxiolytics/Antidepressants (acting on the 5-HT system)

CBT (exposure + trauma narrative), conjoint family therapy (social support); EMDR (Eye Movement Desensitization and Reprocessing YT 1IPsBPH2M1U, 8 steps: history/psychoeducation → desensitization (combining eye movements with trauma narration) → reprocessing/positive cognition → body scan → assessment; breaking the "memory-physiological response" link)

Related Disorder: Acute stress disorder ASD

identical Symptom to PTSD, but 3D~1M duration (it progresses to PTSD if it exceeds 1 month).

Prevalence: 30% following motor vehicle accidents and 30–50% following interpersonal violence; more common in women; approximately 50% eventually develop PTSD.

Other stress and trauma related disorders:

Adjustment disorder (AD) (5~20% prevalence): emotional behaviroal symptom s in response to an identifiable stressor occurring within 3M of the onset of stressor, out of propotional to stress intensity but with significant impairment, usually follow such as losing job or diverse. Symptoms should resolve 6 months after

termination/improvement of the stressor (finding a job, improving relationship/financially)

Prolong Grief disorder: Maladaptive grief reaction extended more than 12M, intense yearning/longing and /or preoccupation with thoughts or memories of the deceased, may hold maladaptive cognition about self and have harmful health behaviors. Exhibits 3+ among{identity disruption ,sense of disbelief about the death, avoidance of remainder, intense motional pains, …}

Mood Disorders 心境障碍

Disturbance in emotions that Cause subjective discomfort, hinder a person's ability to function

Two Types: Depressive (unipolar) and bipolar (difference in mood episodes, major depressive / mania / hypomania)

Major depressive episode (MDE)

Depressed mood, and diminished interest of pleasure (Anhedonia), lasts most of the day, nearly every day for 2W+

Anhedonia 快感缺失: loss of pleasure or interest in usual activities

(Anhedonia OR Depressed mood) AND 5+ out of 9 { Anhedonia, Depressed mood, Significant weight loss/gain or change in appetite, Insomnia or hypersomnia 失眠或睡眠过多, observable Psychomotor 精神运动性 agitation or retardation, fatigue/loss of energy, Feelings of worthlessness or excessive/inappropriate guilt, indecisiveness and difficulty thinking/noting, Suicidal ideation/attempt }

Chain: Depressed mood -> anhedonia -> feeling of worthlessness or guilt -> suicidal ideation / plans

Manic Episode 躁狂发作

Speech becomes {Loud-rapid, pinning, rhyming, clanging, vulgar, pressured speech}, Grandiose delusions (inflated self-esteem or grandiosity), hyperactive, less need for sleep, inappropriate, flight of ideas and racing thoughts, distractibility (attention too easily frown to unimportant or irrelevant), increase in goal directed activity (socially/ at work or school / sexually) or agitation, excessive involvement in high-risk activities (unrestrained buying sprees, sexual indulgence, foolish investment or gambling)

Impairment in normal functioning, excessive involvement in pleasurable but risky behavior, elevated mood almost everyday for 1W+

Example: Mania can resemble schizophrenia or even a crack high, YT: p9hbXPVaOuk

Hypomanic episode 轻躁狂发作: Shorter less severe version of manic episodes (4D+), fewer and milder symptoms

Mixed features: can occur simultaneously or in very short succession, even if opposite diagnostic symptoms (bipolar)

Depressive disorder (unipolar)

Major depressive disorder MDD 重性抑郁障碍

(same criteria to MDE) Depressed mood most of the day, nearly every day, decreased physical activity and reduced productivity (fatigue), negative view of self, difficulty concentration, suicidal ideation, change in eating habit, sleep problems, psychomotor agitation, NEVER been a manic episode or hypomanic episode

Can be described: {mild/moderate/severe} With Anxious distress, mixed features (some mania but not meeting criteria), melancholic features, atypical features, mood-congruent/incongruent psychotic features, catatonia (紧张症 YT:zAEJ-Jvndms), peripartum onset, seasonal pattern

Example: Living Through Depression: Julia's Story, YT: a1Y1ocyudjs

Prevalence: 7% 12M F>M (1.5~3x), highest between 18~ 29 years old, incident peak at 20

Comorbidities: substance-related disorders (use of substances to cope with symptoms), panic disorder, OCD, eating disorders (anorexia, bulimia), borderline personality disorder

Persistent depressive disorder (Dysthymia) 持续性抑郁障碍（心境恶劣）

Similar to MDD, but only 2+ out of 9 syndrome required for diagnosis (can be less severe than MDD). Form of

chronic depression 慢性抑郁症 lasting for 2 years or longer, and NEVER without symptoms for more than 2M

Disruptive mood dysregulation disorder DMDD 破坏性心境失调障碍

Observable childhood condition of extreme irritability, anger, and frequent, intense temper outbursts (between which is persistently irritable or angry most of the day), recurrent and out of proportion, inconsistent with developmental level, 3+ times per week, present for 12+M, consistent between contexts (2+ scenarios), onset before 10 years (never diagnosis to <6 or >18 years old),

Premenstrual dysphonic disorder

Causes severe irritability, depression, or anxiety in the week or two before period starts, combined with physical symptoms, severe mood swing and anxiety

Prevalence: 1.8~5.8% of menstruating woman

People may hide their depressive symptoms or many reasons (stagnation, fear of burdenting others or backlash, embarrassment, denial, concern about appearing weak, unrealistic thoughts about happiness)

Example: Smiling depression, harder to diagnose, 6 Signs of Smiling Depression (Only appear cheerful and optimistic in front of others; Obsessed with creating a "perfect life" on social media; Don't ask for help for fear of appearing weak (men are less likely to ask for help); Force a smile during major life changes (unemployment, immigration, bereavement); Always smile and appear unaffected; Throw yourself into hobbies/work to escape emotions → Emotional and physical exhaustion), YT:r18UBX9rLdk

<u>Bipolar Disorders</u>

Bipolar I disorder: alternation between MDD and manic episode (must meet manic criteria)

Bipolar II disorder: MDD + hypermania (must NEVER diagnosed manic), normally MDD period longer than Bipolar I

Cyclothymic disorder 环性心境障碍: Chronic bipolar form lasting 2+ years, hypomanic-like and depressive-like symptoms, but both do not meet diagnostic criteria, NEVER been symptom-free for more than 2 months.

Rapid Cycling Pattern: ≥ 4 mood episodes in 1 year (depressive, asymptomatic, alternating mania/hypomania), applicable to both I and II

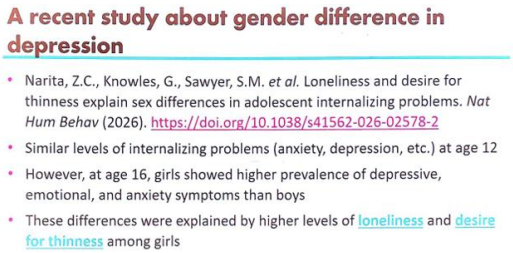
Prevalence

USA 0.6%, China 0.3% for Bipolar I; USA 0.8%, China 0.1% for Bipolar II; Cyclothymia: 0.4%–1%.

no gender difference, but F>M for Bipolar II with rapid cycling

Age of onset: Bipolar I around late adolescence; II around early adulthood; Cyclothymia around early adolescence.

Comorbidities: Anxiety disorders, ADHD, destructive impulse control/conduct disorders, various substance use disorders.



Suicide attempts: unipolar depression 12%, bipolar I 17%, II 24%, likely during depressive episodes,

<u>Etiology</u>

biological perspective: genetics, neurotransmitter (serotonin 血清素, norepinephrine 去甲肾上腺素, dopamine, permissive hypothesis), Endocrine (HPA axis), brain function, Sleep and circadian rhythm, genes–environment

Psychological perspective

Behavioral orientation: Depression = lack of positive reinforcement ("Nothing can make me feel good") →

withdrawal from various activities (operant conditioning: no good results → learning to do nothing).

cognitive orientation: learned helplessness 习得性无助, style of attribution (internal/external, stable/unstable, global/specific), cognitive distortion

Beck's cognitive theory of depression: depressive cognitive triad 抑郁认知三角: Negative views about self (I am a bad person/worthless/loser), the world (no one loves me/likes me), and the future (hopeless, anticipating negative outcomes; Cognitive biases/distortions and negative schemas

Sociocultural Perspective: Social relations/social support, marital relationship, cultural difference (Asians present more with somatic complaints, whereas Westerners present more with psychological complaints), gender difference (artifacts 人为因素, life stress, gender roles, rumination 反刍说)

Treatment

Medication: antidepressant medications (SSRIs, tricyclic antidepressants TCAs, MAOIs) for unipolar, Lithium for bipolar

CBT 4 phases: Improve mood through pleasant activities and formulate concrete action plans -> Psychoeducation -> Teaching patients to monitor automatic thoughts (when negative events occur) -> Identifying these thoughts and how they perpetuate depression, then challenging and replacing them with more adaptive thoughts

Behavioral Activation: Focuses solely on improving mood (identify -ve / +ve behaviors and engaging in social skills training to derive satisfaction), with the goal of maximizing pleasurable experiences while minimizing negative ones

Interpersonal Therapy 人际治疗

Schizophrenia 精神分裂症 Spectrum and other Psychotic Disorder



Spectrums: Schizophrenia 精神分裂症, brief psychotic disorder 短暂精神病性障碍, schizophreniform disorder 精神分裂症样障碍, schizoaffective disorder 分裂情感性障碍, delusional disorder 妄想障碍, schizotypal personality disorder 分裂型人格障碍

Domains of anomality:

positive symptoms 阳性症状 (additional extraordinary experience, including **Delusions** 妄想 and **hallucinations**)

Disorder/Cognitive symptoms (including **disorganized thinking**, **severely disorganized or abnormal behavior**)

negative symptoms 阴性症状

Delusions: Fixed erroneous beliefs, cannot be changed even facing contradictory evidence; detached from reality and unable to be updated based on actual experiences.

Types: bizarre (implausible / impossible contents, e.g., abducted by aliens) and non-bizarre (e.g., abducted by thief)

Possible contents: Grandeur 夸大, persecution 被害, erotomania 钟情, jealous, somatic 躯体, thought broadcasting. Related to one's background

Hallucination: False experiences in the absence of external stimulation

Types: Auditory (~50% of patients), visual (~15% of), olfactory 幻嗅 (rare), gustatory 幻味(rare), tactile(~5%)幻触, can occur in combination,

Example: YT:N6_suRopnC4

Disorganized Thinking: disjointed and incoherent thought processes (visible in speech)

Cognitive slippage, circumstantiality, tangentiality, speech retardation, loose associations/derailment, neologisms

Disorganized/abnormal motor behavior, or catatonic behavior

Negative symptoms: insufficiency or absence of normal behavior in loss of function, including avolition 意志缺乏, apathy 冷漠, alogia 言语贫乏, anhedonia 快感缺失, affective flattening 情感平淡, asociality 社交缺乏

Diagnosis Criteria

Shows 2+ of the 5 domains with at least 1 being {delusions, hallucinations, or disorganized}, for a significant portion of time during a 1M period, or

Level of functioning in one or more major areas, markedly below the level achieved prior to the onset, or

Continuous signs of disturbance persist for 6+M (at least 1M of symptoms of A), prodromal or residual symptoms, manifested by only netative symotoms or by 2+ symptoms

Course of schizophrenia: premorbid -> prodromal -> onset/deterioration -> chronic/residual

Specifiers of schizophrenia: first episode, multiple episodes, currently in, continuous vs residual symptoms accumulating over time

Prevalence: 0.3~0.7% life, chronic course, lower life expectancy rate, gender difference in mixed, onset in earlier for males, normally 20~24 for M and 25~29 for F

Living with Schizophrenia: Management, Support & Hope | WebMD YT:C7Jl9_59tfY

<u>Schizophrenia Spectrum Disorders</u>

Mild -> Severe: Schizotypal personality disorder STPD 分裂型人格障碍, delusional disorder 妄想症, brief psychotic disorder 短暂性精神病性障碍, schizophreniform disorder 精神分裂症样障碍, schizophrenia 精神分裂症, Schizoaffective disorder 分裂情感性障碍 (most severe, adds concurrent major mood episodes)

Brief Psychotic Disorder: At least 1 of {Delusions, hallucinations, disorganized speech}, duration for 1 day to less than 1M, Return to normal level of functioning afterward

Schizophreniform Disorder: Similar to schizophrenia, at least 1M to max 6M (otherwise to schizophrenia)

Schizoaffective disorder: Dominated psychotic symptoms + concurrent major mood episode, persist at least 2M, 0.3 lifetime prevalence, F>M

Delusional Disorder: difficult to discern/distinguish between external reality and one's false belief, disconnection between ext and int world, for 1+M, not suffer from disorganized thinking, may have hallucination but are not dominant, ~0.2% lifetime without gender difference

Causes

Biological Perspective

Genetic inheritance: family studies, twin studies, adoption studies, influenced by over 250 genes

Neurotransmitter: dopamine, glutamate 谷氨酸, and serotonin

reduced function of dopamine D1 receptor in prefrontal cortex -> negative symptoms and cognitive symptoms

Hyperactivity of dopamine D2 receptor in subcortical and limbic brain regions -> positive symptoms

Less activity in dorsolateral prefrontal cortex (DL-PFC), responsible for working memory, motor planning, retrieving useful information

Network dysfunction: auditory hallucination, delusion, reduced caudate and thalamus volume for schizophrenia

Change in brain structure: ventricles, shrinkages

Gene-env interaction (diathesis-stress model 素质-压力模型): genetic/biological predisposition + stressful env = risk of schizophrenia,

Prenatal and perinatal influences: threat to neurodevelopment

Psychological perspective:

Psychodynamic approach:

symptoms as a defense against unbearable aspects of reality (can be with a weak ego and/or harsh ego),

symptoms are meaningful in terms of personal info disguised to varying extents in hallucinations and delusions

Cognitive approach

Dysfunctional attitude and maladaptive cognitions (cognitive distortions)

Neurocognitive impairment <-> aversive life events -> dusfunctional atitudes -> cognitive distortions -> HPA -> schizophrenia

Sociocultural perspective: family env (disturbed family env, poor communication patterns, expressed emotion), cross cultural differences

Treatment

Medications: antipsychotic medications: control symptoms of schizophrenia and reduce/eliminate positive symptoms, 2nd-generation agents deal with negative symptoms

- Reasons for non-adherence are complex, some possible reasons are
  - personal factors such as poor insight
  - health beliefs such as refusal of medication
  - drug factors such as poor efficacy
  - drug-related side effects
  - medication interference with life goals
  - social factors such as embarrassment/stigma over illness and lack of social support

Psychotherapy:

CBT: to improve the interpretations and understandings of the symptoms and experiences, not to reduce the symptoms on its own, psychoeducation, cognitive reconstruction (believe modification for delusions, focusing and reattribution for hallucinations, normalization of symptoms and reducing negative self-stagnation), coping with stress/anxiety via relaxation

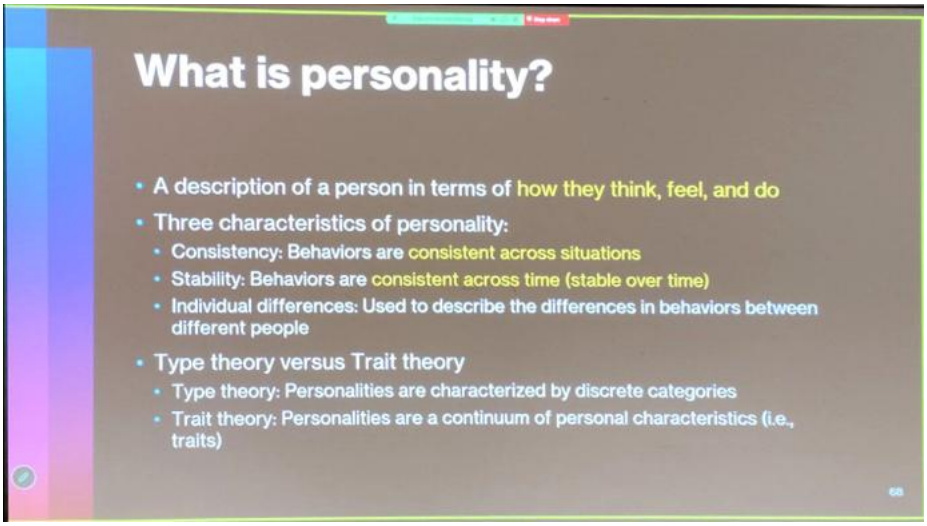
Social skill training and independent living skills

Family therapy YT: nRTYg5n5Bq0

Some notes on Quiz

- No percentages or onset ages (e.g., prevalence, onset age)
- No year or name
- No comorbidity
- No details of medication (mechanism + side effects); you only need to know which type of medication is for treating which disorders (e.g., SSRIs for anxiety and depression; Lithium for bipolar disorder, etc.)
- No exact DSM statements (e.g., how many symptoms are needed to fulfill the diagnostic criteria); you only need to know what the features of the disorders are and how they are different from each other (the defining features of the disorder) - understanding is required but not exact wording from DSM

Personality Disorder 人格障碍



Personality disorders: when personality trains are inflexible and maladaptive and abuse significant functional impairment or subjective distress do they constitute personality disorders,

Deviates markedly from individual's culture in 2+ of {cognition, affectivity, interpersonal functioning, impulse control}, stable and of long duration and onset can be traced by adolescents

3 Clusters: A: odd or eccentric, B: dramatic emotional erratic, C: anxious or fearful

Cluster A Disorders

Paranoid Personality Disorder 偏执型人格障碍

distrust and suspicion: Believe that other people have a negative intention against oneself (despite having no proper reason and evidence), very critical of others and argumentative, interpret unintentional actions as intentional, easily get irritated and angry, unwilling to forgive, rigidity in belief, creates problems in interpersonal relationships

YT:drzKThkBLOc

Diagnostic: 4 out of 7 among {exploting harming or deceiving, preoccupied with unjustified doubts reluctant to confide in others, read hidden demaining or threating meaning, persistently bears grudges, perceives attacks on one's character or reputation, recurrent suspicion}

Schizoid Personality Disorder SzPD 分裂**样**人格障碍

Detachment and isolation: lacking of interest in social relationships (not desire nor enjoy close relationships), sexual behavior, pervasive pattern of detachment, emotional coldness/flatten in social situations, indifference to praise and criticism, prefer being and doing things along, but not psychotic nor disorganized thought

Solitude != schizoid personality disorder

Schizotypal personality disorder STPD 分裂**型**人格障碍

Odd cognitive perceptual distortions and eccentric behaviors: idea of reference (thinking unrelated events are related to oneself), perception of special or magical powers, unusual perceptual experience, unusual speech patterns

discomfort with/reduced capacity for close relationship: lack of close friend, paranoid ideation, restrictive or inappropriate affect -> reduced ability to interact with others in social contexts, strong social anxiety when interacting other than 1st degree relatives, socially inappropriate appearance or behavior

Epidemiology (Cluster A):

Prevalence: schizoid 3.1–4.9%; paranoid 3.3–4.4%; schizotypal 0.6–3.9%.

Gender: schizoid & schizotypal more common among males.

Comorbidity: co-occurs with mood disorders, anxiety disorders, OCD, substance abuse/dependence

Cluster B

Antisocial Personality Disorder

Rule breaking and violating the rights of others, no moral compass, not feeling remorse over their misdeeds, Lacking empathy, Impulsive and irritable, recurrent illegal act lies fights with others, failure to take responsibility, sometimes aka psychopath or sociopath

Must be at least 18 years old, evidence of conduct disorder with onset before age 15 years, not exclusively during course of schizophrenia or bipolar disorder

Example: Ted Bundy YT 9rqNPVByC9g, agreed to has at least 2 PDs in 2007: antisocial and narcissistic

Borderline Personality Disorder 边缘性人格障碍

5+ out of 10 among {Instability in relationship, self-image, affect, avoid real or imagined abandonment, impulsive behaviors (gamble, binge eat, risky sex, drug abuse), potentially self-harm, recurrent suicidal behavior, chronic feelings of emptiness, trouble in controlling anger, transient stress-related paranoid}

Can have transient psychotic symptoms

May feel more secure with pets/inanimate objects than interpersonal

Histrionic Personality Disorder 表演型人格障碍

Excessive attention-seeking and emotionality,

5+ out of among {strong need to be the center of attention (uncomfortable if not), talk loud but may meaningless, inappropriately sexually provocative or seductive in social contexts, overly concerned about impressing others or getting approval (by makeups or costume but may inappropriate), exaggerate expression of emotions and rapid change of emotion, easily influenced by others, consider relationship to be more intimate than it is (may manipulative in interpersonal relation)

Narcissistic Personality Disorder 自恋型人格障碍

Grandiosity, excessive need for admiration, and lacking empathy

Overestimate abilities and accomplishments, ruminate about long overdue admiration and privilege, compare themselves favorability with famous or privileged people, belief of superior and unique, think that their need is special and different from ordinary people, fragile self-esteem, self-focus, sense of entitlement and low sensitivity to other's need -> exploitation of others, easily feel envious toward others, intolerance of others' critic but tendency to belittle others

Prevalence: Antisocial Personality Disorder 3.3% Borderline Personality Disorder 5.9% Histrionic Personality Disorder 1.8% Narcissistic Personality Disorder 6.2%

Cluster C: Anxious and Fear

Avoidant Personality Disorder 回避型人格障碍: Cowardly

Excessive social anxiety (inadequacy and inferiority, thinking of expected negative criticisms, rejections,

disapproval), anticipated negative emotions, tend to be shy/quiet/inhibited, avoidant in occupational activities that involve significant interpersonal contact, unwilling to get involved with people unless curtained of being likes, restraint within intimate relation, inhibited in new interpersonal situations, socially inept personally unappealing or inferior to others, unwilling to take personal risk

Dependent Personality Disorder 依赖型人格障碍: Clingy

Unable to function without other's help, low confidence, and difficult in making decisions by oneself, fear of losing support, needs others to assume responsibility, difficulty expressing disagreement, difficulty initiating projects or doing things

Obsessive-Compulsive Personality Disorder

4+ out of 8 among: Excessive obsession with rules/lists/schedules and order to maintain a sense of control (extraordinary attention to detail, poor time allocation), excessive perfectionism (every detail to be perfect -> tasks cannot finish), (excessive devotion to work:) {very uncomfortable for leisure or vacations, reluctant to delegate jobs}, excessive conscientiousness (scrupulous and inflexible), (excessive keeping for future use:) {unable to discard things, tightness in keeping money}, shows rigidity and stubbornness

OCD is not rigid in personality, not usual orderliness in general , control, perfectionism, indicisiveness, with in trisive thoughts behaviors, need to perform acts, recognition of irrationality

Prevalence: Avoidant PD 2.4%, Dependent PD 0.5~0.6%, OCPD 2.1~7.9%, Dependent PD F>M, OCPD M>F

Etiology

Biological Perspective: Weak to moderate degree of inheritability

Schizotypal PD: (similar to schizophrenia) high activity of dopamine and enlarged brain ventricles

Antisocial PD and Borderline PD: deficits in serotonin, functioning of the frontal lobes: regions for planning, self-control

Arousal Hypothesis for antisocial PD: lacking of physiological arousal towards cues of punishment pain, and anticipated danger

Psychodynamic approach

Early childhood experience, higher levels of childhood neglect and parental rejection, negativity in childhood -> perception of the self and the other

Cognitive approach

Biological vulnerability + environmental stressors + maladaptive cognitions

Distorted cognition and maladaptive thought patterns -> inaccurate perception, can be learned during childhood

Behavioral approach: Modeling: learning maladaptive patterns of behaviors from parents/family members

Sociocultural perspective: Family dysfunction, childhood maltreatment

Treatment

Difficult but not impossible: personality defines a person

People may not actively seek for treatment: Antisocial, narcissistic, paranoid, schizoid PD

Cluster A: CBT. STPD – antipsychotic

Cluster B: CT, Antidepressant, antipsychotics, mood stabilizers, dialectical behavior therapy (DBT, one form of CBT, including skills training, enhancing motivation, telephone and in vivo coaching, case management, consultation team. E.g., Q8H5KqBagnI)

Cluster C: CT or CBT

Somatic Symptoms and Related Disorders

Somatic Symptoms (formerly "somatoform disorder): Symptoms considered to be authentic, negative appraisal of symptoms (considered unduly worrying), for 6M+

Illness anxiety disorder (formerly "Hypochondriasis"): excessive preoccupation with having/acquiring a serious illness, typically have no physical symptoms or have only mild and normal, excessive worry and care-seeking (really)/avoidance. E.g., medical student https://www.cbsnews.com/video/hypochondria-why-medical-students-and-many-others-worry-about-conditions-they-dont-really-have, YT:H9Flt7CFAZY

Conversion disorder (aka functional neurological symptom disorder): Loss of voluntary motor or sensory functioning resembling a neurological condition, no known medical/organic causes, not consciously produced or feigned, usually precedes by psychological stressors/distresses , YT:cIzPnCIqxng

Factitious disorder: exaggerate fake simulate or induce physiological or psychological to obtain a , but no malingering motivation (falsifying illness to gain external benefits such as avoiding work/trial/military or seeking drugs, symptoms will disappear after satisfying). E.g., YT:v-ZOB1sHLr8

Prevalence: Somatic Symptoms 5~7% Illness anxiety disorder 1~10% Conversion disorder 0.11~0.5% Factitious disorder 0.5~2%,

Etiology

unconscious emotional issues: primary gain (internal motivator), secondary gain (external motivator)

Cognitive approach: Negative beliefs or exaggerated fears,

Behavioral approach: attention and sympathy as reinforcement

Sociocultural Childhood trauma as a risk factor, social influence, mind-body interaction by eastern culture

Treatment

Interpersonal Psychotherapy: address the innate goal to maintain interpersonal relationships, secure attachment, improve the quality of interpersonal relationships and social functioning

CBT: psychoeducation, address cognitive misattribution and maladaptive coping strategies, empowerment, corrects avoidance behaviors, mediates expectations of treatment

Dissociative disorder

Disruption or discontinuity in the normal integration of consciousness, memory, identity, etc.

Dissociative Identity Disorder DID (aka. Multiple personality disorder): Alterations/discontinuities in identity, depersonalization ("out-of-body" experience), amnesia, dominant personality and subpersonalities

Dissociative Amnesia Disorder

Depersonalization Disorder and Derealization Disorder